

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	738
	A Division of the Merced County Department of Public Health	Effective Date	11/2025
Authority: Health and Safety Code, Division 2.5, and California Code of Regulations, Title 22, Division 9, Chapter 4, 1797.220		Initial Date:	10/2011
		Review Date:	04/2028

NON-TRAUMATIC SHOCK	
Physical signs may include collapsed peripheral/neck veins, confusion, cyanosis, disorientation, thready pulse, pale/cold/clammy/mottled skin, rapid respirations, and anxiety. Signs of compensation may be absent in the elderly or patients taking beta-blocker or alpha-blocker medications. REMEMBER A DECREASED BLOOD PRESSURE IS A LATE SIGN OF SHOCK.	
ADULT	PEDIATRIC
BLS	
Assess Vitals / Obtain SpO ₂ / Oxygen - Titrate to SpO ₂ of 94% or higher / Secure Airway - Assist Ventilations if needed. Blood Sugar Check – If ALOC.	
ALS	
Follow BLS procedures if applicable. Obtain 4-Lead ECG. Capnography – Utilize Waveform ETCO ₂ .	
Vascular Access – Establish two (2) large bore IVs - If the patient has a SBP < 90 mmHg, administer 500 ml fluid boluses to SBP > 90 mmHg - Reassess patient after each bolus assessing for signs of fluid overload	Vascular Access – Establish large bore IV - If the patient is hypotensive per length-based assessment tape or application, administer 20 ml/kg fluid bolus - Reassess patient after each bolus assessing for signs of fluid overload - May repeat 10 ml/kg bolus to a max 40 ml/kg
<u>If patient maintains hypotension and does not respond to previous treatment:</u> Push Dose Epinephrine - 10mcg slow IV/IO (10mcg/ml) Repeat every 1-5 minutes until hypotension resolves	

Signatures on File

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